

1. Administrative & Study Identification

Full Study Title: TACTIVE-U: AN INTERVENTIONAL SAFETY AND EFFICACY PHASE 1B/2, OPEN-LABEL UMBRELLA STUDY TO INVESTIGATE TOLERABILITY, PK, AND ANTITUMOR ACTIVITY OF VEPDEGESTRANT (ARV-471/PF-07850327), AN ORAL PROTEOLYSIS TARGETING CHIMERA, IN COMBINATION WITH OTHER ANTICANCER TREATMENTS IN PARTICIPANTS AGED 18 AND OLDER WITH ER+ ADVANCED OR METASTATIC BREAST CANCER, SUB-STUDY C (ARV-471 IN COMBINATION WITH SAMURACICLIB) **Short Title/ Acronym:** TACTIVE-U (Sub-Study C) **Protocol Number:** C4891024 **NCT Number:** NCT06125522 **Sponsor Name:** Pfizer in collaboration with Arvinas and Carrick Therapeutics **Investigational Product:** Vepdegestrant (ARV-471) and Samuraciclib (CT7001) **Phase of Development:** Phase 1b/2 **Trial Status:** Active, not recruiting **Medical Monitor:** Pfizer / Arvinas Clinical Operations

2. Study Synopsis & Rationale

2.1 Study Objective Primary Objective: To evaluate the safety, tolerability, pharmacokinetics (PK) including drug-drug interactions, and antitumor activity of vepdegestrant (ARV-471) in combination with samuraciclib, and to determine the Recommended Phase 2 Dose (RP2D). **Secondary Objectives:** To evaluate overall safety, tolerability, preliminary antitumor efficacy, and the Duration of Response (DoR) of the combination regimen.

2.2 Background & Rationale Vepdegestrant is a novel PROTAC (PROteolysis TARgeting Chimera) estrogen receptor degrader, and samuraciclib is a first-in-class CDK7 inhibitor. This study addresses the medical necessity of finding effective treatment combinations for patients with Estrogen Receptor Positive/Human Epidermal Growth Factor Receptor 2 Negative (ER+/HER2-) advanced or metastatic breast cancer whose disease is no longer responding to prior therapies, particularly previous CDK4/6 inhibitor treatments.

3. Study Design & Methodology

3.1 Design Framework Study Type: Interventional **Control Type:** Single-arm (Sub-Study C within the TACTIVE-U Umbrella platform) **Blinding:** Open-label **Randomization:** N/A (Phase 1b utilizes a dose escalation/de-escalation approach based on mTPI-2 decision criteria)

3.2 Planned Enrollment & Duration Target **N**: 11 participants
Number of Sites: Multicenter global study (including sites in the United States, Belgium, France, and Italy) **Estimated Study Start**: February 2024 (First patient dosed) **Estimated Primary Completion**: Ongoing; duration based on RP2D determination and efficacy endpoints

4. Subject Selection (Eligibility Criteria)

4.1 Inclusion Criteria 1. Age ≥ 18 years with a histological or cytological diagnosis of ER+/HER2- breast cancer that is advanced or metastatic and not amenable to curative surgical resection ($\geq 1\%$ ER+ stained cells). 2. Prior anticancer therapies: up to 2 lines of prior therapies for advanced/metastatic disease; patients must have received exactly 1 line of a CDK4/6 inhibitor-based regimen. 3. At least 1 measurable lesion as defined by RECIST v1.1 and an ECOG Performance Status of ≤ 1 .

4.2 Exclusion Criteria 1. Visceral crisis at risk of life-threatening complications in the short term. 2. Known history of drug-induced pneumonitis or other significant symptomatic deterioration of lung functions; newly diagnosed brain metastases, symptomatic CNS metastases, carcinomatous meningitis, or leptomeningeal disease. (Treated, clinically stable CNS metastases are permitted if medications are discontinued for ≥ 28 days). 3. Impaired cardiovascular function, clinically significant cardiovascular diseases, or concurrent administration of medications/food/supplements that are strong CYP3A inhibitors/inducers, strong CYP2D6 inhibitors, or drugs predisposing to Torsade de Pointes or QT prolongation. 4. History of any other tumor malignancies within the past 3 years, except for adequately treated basal or squamous cell carcinoma of the skin, or curatively treated in situ carcinoma of the cervix. 5. Inflammatory breast cancer. 6. Renal impairment, inadequate liver function and/or bone marrow function, or known active infection.

5. Intervention & Treatment Plan

5.1 Investigational Regimen Dosage/Frequency: Vepdegestrant and samuraciclib are both administered orally once a day (QD) continuously. Phase 1b tests escalating dose combinations to establish the RP2D. **Route of Administration**: Oral **Duration of Treatment**: Administered continuously in 28-day cycles until disease progression or unacceptable side effects.

5.2 Concomitant & Prohibited Therapy Permitted: Standard supportive care measures and medications for comorbidities. **Prohibited**: Strong CYP3A inhibitors/inducers, strong CYP2D6 inhibitors, and drugs known to prolong the QT interval. Anti-seizure medications and corticosteroids must be discontinued for

at least 28 days prior to enrollment if treating stable CNS metastases.

6. Study Timeline & Visit Schedule

Visit ID	Window (Days)	Key Activities/Procedures
Screening	Day -28 to 0	Informed Consent, Medical History, Tumor Assessment (RECIST v1.1), Baseline Labs
Baseline	Cycle 1 Day 1	First doses of vepdegestrant and samuraciclib, Baseline PK profiling
Interim	Every 4 weeks	Clinic visits for safety monitoring, PK sampling, AE review, and tumor efficacy assessments
End of Study	Follow-up	Treatment discontinuation due to disease progression or severe toxicity, Final evaluations

7. Outcome Measures (Endpoints)

7.1 Primary Endpoints

- Dose Limiting Toxicities (DLTs):** Number of participants with DLTs for ARV-471 in combination with Samuraciclib estimated based on data from DLT-evaluable participants during Cycle 1 to determine RP2D.
- Drug-Drug Interaction (AUC_{tau}):** Steady-state Area under the plasma concentration versus time curve (AUC_{tau}) of ARV-471 with and without coadministration of samuraciclib.
- Drug-Drug Interaction (C_{max}):** Steady-state Peak Plasma concentration (C_{max}) of ARV-471 with and without coadministration of samuraciclib.

7.2 Secondary & Exploratory Endpoints

- Evaluation of Safety and Tolerability of ARV-471 in combination with samuraciclib.
- Evaluation of antitumor activity of ARV-471 in combination with samuraciclib.
- Duration of Response (DoR) by investigator assessment.

8. Safety & Adverse Event Reporting

Adverse Event (AE) Monitoring: AEs will be characterized by type, frequency, intensity, timing, and relationship to the investigational product combo using NCI CTCAE version 5.0.

Serious Adverse Events (SAEs): Captured throughout all cycles of treatment and evaluated for DLT criteria.

Data Monitoring Committee (DMC): A safety review committee oversees the dose escalation/de-escalation guided by the mTPI-2 decision framework.

9. Statistical Analysis Plan (SAP) Summary

Analysis Populations: Safety Set (all treated participants evaluating AEs/DTs); Efficacy Set (participants with RECIST-

measurable disease). **Sample Size Justification:** The targeted sample size of 11 is driven by the dynamic mTPI-2 decision framework rules for escalation/de-escalation based on the number of DLT-evaluable subjects in Phase 1b. **Handling of Missing Data:** Follows standard oncological statistical conventions (e.g., right-censoring for time-to-event outcomes).

10. Quality Assurance & Regulatory Compliance

GCP Compliance: This study will be conducted in accordance with Good Clinical Practice (GCP) and the Declaration of Helsinki.

Monitoring Plan: Standard clinical site monitoring with 28-day cycle evaluations to track patient safety and compliance. **Audit**

Trail: Robust centralized data tracking with eCRF locks, particularly critical during the mTPI-2 dose escalation phase.

11. Study Identifiers & Governance

Primary ID: C4891024 **Registry Number:** NCT06125522 **Posting ID:**

774193812361215864 **Sponsor/Lead Organization:** Pfizer, Arvinas, Carrick Therapeutics **Study Title:** TACTIVE-U (Sub-Study C)

Official Regulatory Title: TACTIVE-U: AN INTERVENTIONAL SAFETY AND EFFICACY PHASE 1B/2, OPEN-LABEL UMBRELLA STUDY TO INVESTIGATE TOLERABILITY, PK, AND ANTITUMOR ACTIVITY OF VEPDEGESTRANT (ARV-471/PF-07850327), AN ORAL PROTEOLYSIS TARGETING CHIMERA, IN COMBINATION WITH OTHER ANTICANCER TREATMENTS IN PARTICIPANTS AGED 18 AND OLDER WITH ER+ ADVANCED OR METASTATIC BREAST CANCER

12. Clinical Framework (Breast Cancer Specific)

Disease Area: Breast Cancer **Primary Condition:** ER+/HER2- Advanced or Metastatic Breast Cancer **Diagnosis Threshold:** ER+ ($\geq 1\%$ stained cells by IHC) and HER2- **Medical Methodology:**

Investigational combination of an oral PROTAC estrogen receptor degrader and a novel oral CDK7 inhibitor **Optimization Target:**

Dose optimization (RP2D) and tumor progression control

13. Study Procedures & Methodology

Management Pathway: Continuous 28-day cycle therapy until cancer progression or severe side effects **Education Protocol (HCP):** Site training on dose-escalation management and mTPI-2 toxicity criteria **Education Protocol (Patient):** Directions for daily at-home oral dosing of vepdegestrant and samuraciclib **Quality Audit**

Mechanism: Clinical and laboratory AE reviews utilizing CTCAE v5.0 every 4 weeks

14. Observation & Follow-Up Schedule

Total Duration: Continuous monitoring until disease progression or toxicity **Onsite Visit Cadence:** Approximately every 4 weeks
Remote Monitoring Frequency: Between cycles as medically necessary for AE management **Checklist Review Interval:** Regular tumor assessments per RECIST v1.1 to evaluate efficacy and DoR

15. Sign-off & Approval

Role	Name	Signature	Date		----	----	----	----
Principal Investigator	[Name]	_____	[DD-MMM-YYYY]					
Clinical Operations Lead	[Name]	_____	[DD-MMM-YYYY]					Lead
Statistician	[Name]	_____	[DD-MMM-YYYY]					